

Notes on Grievance hearing 03Nov25

Present: Suzanne Kennedy (SK), Rehan Minhas (RM), Daniel South (note taker)

(Notes typed up on 03Nov25, from handwritten notes taken at the time of the meeting)

SK explained the reason for the meeting. Grievances raised include public humiliation, sharing of health information, unreasonable target setting. SK has reviewed the grievance letter, submissions and redacted interviews. Events began in wk3 July. SK acknowledged that information submitted is comprehensive and in chronological order.

SK asked what RM likes about the role.

RM likes that Lonza and the role gives the opportunity to contribute to patient safety and have a purpose. Works as a biotechnologist in manufacturing. Is a GMP environment and works with many cell lines. Completed training and then worked in buffer preparation before moving to manufacturing suites. SK asked to break down his role more specifically. RM clarified he joined downstream purification team to work in purification, handling product and buffers. Had worked in a lab-based role up until now. Has experience of regulated environment. Teamwork also plays a large role. Plant gives a different environment. Is a hands on role. Gets to build on previous experience. Learning about purification. Role has met expectations to date.

SK asked what RM hopes outcome of the meeting will be. RM responded he wants the grievance to be upheld, some corrective action to avoid repetition of event and compensation for impact.

SK: What events caused suspicions data was not being stored correctly?

RM: Had probation review and shared details of health issues and medication being taken. Asked for confidentiality. A few days later was shown a Excel file that contained details of mental health issue saying this led to RM looking nervous. This was detailed against RM's name. RM wanted to keep this private. He let his line manager know but didn't want to share further.

On 26Sep25 was in smoking area with another plant lead who asked about RM's health. Conversation moved to 'Are you able to do the job?'. Felt competency was being questioned. This was after the health disclosure to line manager. No suspicions for data breach other than information recorded in Excel sheet held by line manager.

SK: Referring to public humiliation and repeated questioning. What about the questions were intrusive?

RM: Had to leave plant to use the toilet. Had diarrhoea on 2 occasions and couldn't return to plant in case needed to use the toilet again. On 2 other similar occasions was able to return to plant after suffering diarrhoea. Have to use toilet 2-3 times per shift normally. Had less control of this due to medication. After 2-3 times this was noticed by colleagues who asked repeatedly if he was okay. Asked line manager if he could stop asking this and he (RM) would raise if he was not okay. Asked 16 times in one day on one occasion. But asked multiple times every day. One occasion had to leave plant to use toilet. Informed supervisor. While he was away the supervisor informed line manager as he was concerned about a young guy needing the toilet.

Later on, the line manager came to plant and spoke to RM in the change area. Asked 3 times if he was okay.

SK: Was there an indication of absence on plant that would be noticed in the work?

RM: Didn't think so. Was concerned that being told repeatedly by managers 'Are you okay?' was too much. Being asked this in hot desk area gave negative impression to the team.

SK: Which probation targets were unrealistic?

RM: Usual target for CSOD completion 3 months post start is 90%. His target was 95%. Other team members said there wasn't targets for CSOD completion as part of probation. Didn't understand what his was 95%. Was doing his best to deliver role that was impacted by health issue. Feels this issue caused everything to have a negative atmosphere. Feels probation was used as tool to put him under scrutiny and apply pressure.

SK: Were other new starters at same time given CSOD completion targets?

RM: Didn't say this. Was given tasks related to buffer prep in MFG suites as part of probation. As well as CSOD completion.

SK: Role and training plans are in CSOD. 95% is general goal across site to maintain. What did you feel was discriminatory?

RM: Felt it was excessive compared to peers probation targets. Others had different targets.

SK: Can clarify if targets themselves were discriminatory or what were asked to do outside of the targets?

RM: Instruction booklet says CSOD target is 90%. Was also target to be signed off in buffer prep. Peers said they didn't have these targets.

SK: What led to ask others about targets?

RM: Asked others how long they thought it would take to get signed off in training activities. Asked other new starters.

SK: Did you reflect on why asking other new starters how long training would take?

RM: Was motivated to complete training. Most of other people in suites were trainees so asked others going through the same process.

SK: What was your expectation after following UKSL-425 disclosures? Which is a process intended to protect product.

RM: Have to make reports under this SOP. Would have made sense to raise Occupational health assessment following disclosure.

SK: Can't see requirement to report to OH in UKSL-425. Did you request this or discuss with your line manager?

RM: Line manager went on annual leave shortly after joining for about a month. During this time took one day sick leave and did return to work interview with another manager. During this answered that was still taking medication so OH referral was raised and had an assessment meeting. Not sure of outcome or if there was a report. Didn't request an OH referral later as was managing condition.

SK: Understanding you were completing UKSL-425 which isn't a formal OH process, and you did have an OH meeting, what is the grievance issue?

RM: SOP says to disclose diarrhoea and this is supposed to be sent to OH. Had another incident recently where had an eye infection. Completed form in this case which did have an OH referral and an OH doctor signed back on plant. Feels OH should have assessed the other diarrhoea incidents.

SK: Can you describe the pressure to work on plant?

RM: Comments and questions such as 'Are you okay?', 'Do you feel okay to go back on plant?'

SK: What forms the basis for grievance?

RM: Have incontinence issue due to medication. Getting off plant takes 15 mins each way. This creates pressure to do job. Obtained GP letter which said needed allowances. Not forced to go on plant but asked repeatedly if was okay to go back on plant. But had incontinence issues and doesn't think it makes sense to not include OH to manage this. Told could be on plant if feeling better and felt pressure to do so.

SK: Were you expecting to work in a cleanroom environment for role?

RM: Yes, I was.

SK: Understand you reached out to peers for feedback. Can you explain what you wanted to gain from feedback?

RM: Wanted feedback to improve performance and provide evidence for probation review. All those asked were supervisors. Wanted pointers on what was doing well or not well.

SK: There people were trainers. Were you not getting feedback as part of training?

RM: Did get feedback from training but was looking for additional general feedback. Common at previous workplace.

SK: Did you ask if this was common at Lonza?

RM: Not really.

SK: thank you for sharing symptoms. Which were exacerbated by experience?

RM: Loss of sleep, restlessness, anxiety and distress. Felt stable before starting but deteriorated after joining.

SK: Is there anything you would like to add?

RM: When joined was trying best to do well. Didn't realise incontinence issue was medication induced to start with. So had to get head around that. Pressure with probation targets then started. Line manager raised that people in buffer prep thought he looked nervous and raised health issue most days.

SK: To clarify, do you have anything to add not already covered during process.

RM: Nothing else new to add.